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Mental Health *at Scale*

Amwell vs. Teladoc in the Digital Delivery of Mental Health



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Executive Summary *(Landscape)*



- Digital mental health is clinically effective but limited in scope.
- Teladoc and Amwell follow fundamentally different clinical and business models.
- We identify Amwell as the more sustainable long-term player.

Verdict: Amwell

Reimbursement Matters

Clinical Limits

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Executive Summary

(Verdict)

- We identify Amwell as the more defensible long-term model because it is built on standardized, evidence-based care that aligns with payer reimbursement systems.
- In contrast, Teladoc's BetterHelp relies on unstructured, therapist-dependent care, which limits clinical consistency and reimbursement scalability.
- While Teladoc leads in consumer scale today, Amwell is better positioned for a healthcare system that increasingly values measurable outcomes and integrated care.

Consumer Scale Model

Bottom Line

Clinical Infrastructure Model



Bet on Clinical & Payer Alignment

Why Amwell

- Strong clinical evidence (RCT-backed)
- Standardized care → measurable outcomes
- Better fit for reimbursement systems

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The Mental *Health Crisis*



Why Mental Health Is Hard to Deliver



Mental health demand is large and persistent! ! !

Overwhelming Demand
23% of U.S. adults experienced mental illness in 2024. 21M had a major depressive episode. (NIMH, 2025)

Supply cannot scale with demand! ! !

Workforce Shortage
163M Americans live in shortage areas. HRSA projects a 20% decline in psychiatrists by 2030.

Most patients never enter continuous care! ! !

Treatment Gap
28M adults with mental illness received no treatment. only 27% of need is met in shortage areas.

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Three Clinical Gaps in *Mental Health Care*



Therapeutic Access

46% of psychologists report no availability for new patients
Stigma and scheduling barriers limit in-person care.

Measurement-Based Care

Most in-person care does not systematically track outcomes.

The Severity Gap

Mild cases consume limited clinical resources. Severe patients (e.g., suicidality) require intensive, in-person care

Digital mental health can improve access and standardization, but it cannot fully address clinical complexity, especially for severe conditions.

Benefits vs Limitations of *Digital Mental Health*

Benefits

Clinically Effective (for mild–moderate): RCT evidence shows non-inferiority to in-person care

Improves Access: On-demand, remote, reduces stigma

Enables Measurement-Based Care: PHQ-9 / GAD-7 tracking and outcome monitoring



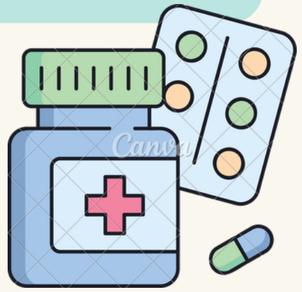
Limitations

Not Suitable for Severe Conditions: (e.g., suicidality, psychosis)

Weak Crisis Management: Hard to intervene in real-time emergencies

Limited Clinical Standardization: Therapist-dependent care varies in quality

Digital mental health expands access and standardization, but cannot fully replace in-person care — especially for high-risk patients.



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From COVID-era boom to post-boom shakeout

2020–2022: The Boom

- COVID-19 triggers 38× increase in telehealth use
- BetterHelp grows from ~400K to 2M+ users
- Teladoc acquires Livongo for $18.5B
- Amwell IPOs at $30/share, peaks at $47

2023–2024: The Reckoning

- Teladoc writes down $13.4B of Livongo goodwill
- Cerebral under DOJ investigation
- FTC fines BetterHelp $7.8M for privacy violations
- Amwell stock falls 47% (pre-reverse-split)

2025–2026: The Restructure

- Teladoc pivots BetterHelp to in-network insurance + acquires UpLift ($45M)
- Amwell divests non-core assets, focuses on SaaS platform
- Targets cash-flow break-even in Q4 2026

Key sub-segments of digital mental health

- Synchronous video therapy (BetterHelp, Talkspace, LifeStance)
- Asynchronous messaging therapy
- Digital CBT / iCBT programs (SilverCloud, Woebot, Meru Health)
- Psychiatric medication management (Cerebral, Done, Talkiatry, UpLift)
- Enterprise / payer platforms (Amwell, Spring Health, Lyra)

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Teladoc / *BetterHelp*

Scale & Model

- FY2025 revenue: $2.53B (-2% YoY)
- BetterHelp: $950M (-9% YoY)
- ~38,000 licensed therapists
- Dual model: Integrated Care + D2C BetterHelp

Technology

- AI matching + messaging + video
- 2025 UpLift acquisition ($45M) → adds 100M covered lives for insurance
- Ambient documentation & session summarization

Main Issues

- 5% user decline + revenue drop in 2025
- FTC $7.8M privacy fine (2023)
- Insurance pivot still <10% of revenue
- High therapist quality variance

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Amwell / *SilverCloud*

Scale & Model

- FY2025 revenue: $245M (-22% YoY)
- B2B SaaS platform + licensed clinical programs
- Targeting cash-flow break-even Q4 2026
- $219M cash

Technology

- Converge multi-tenant SaaS platform (Epic & Cerner integration)
- SilverCloud iCBT programs (6-8 weeks)
- PHQ-9/GAD-7 measurement-based care + real-time dashboards
- AI program personalization & supporter optimization (2025)

Clinical Evidence

- 30+ published RCTs
- NICE (UK) endorsed for anxiety & depression
- Deployed by UK NHS + Irish HSE (55,000+ referrals)
- Up to 80% symptom improvement

Main Issues

- Revenue declined 22% in FY2025
- Lost major DoD behavioral health contract
- Long B2B sales cycles (12–18 months)
- Limited direct-to-consumer brand awareness

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Two Models, *Compared*



- Model: B2C cash-pay vs B2B SaaS
- Payer: Consumer vs Employer
- Evidence: Scale vs 30+ RCTs
- Revenue: $2.53B vs $245M
- Regulatory: FTC fine vs clean

Revenue Trajectory

Clinical Evidence

Business Model

Regulatory Scar

Who Pays

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Risk & Downside

For Patients

Privacy breach: BetterHelp $7.8M FTC fine for sharing MH data with Facebook, Snapchat, Criteo, Pinterest.

For Providers

BetterHelp therapist compensation well below private-practice rates with limited caseload control.

For Payers & Investors

- Over-utilization risk in cash-pay → insurance transition.
- Teladoc market cap fell ~$40B (2021) → ~$1.5B (Apr 2026).
- Amwell did a reverse stock split. Both dependent on policy tailwinds.

For Society

- Digital divide: rural, older, and low-income patients adopt telehealth less.
- Algorithmic bias in matching could entrench disparities in care access and quality.

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Competitive & Landscape

vs. Teladoc/BetterHelp

Talkspace (publicly traded, in-network expansion); Cerebral; LifeStance (hybrid); Brightside; Ginger / Headspace Health. Direct B2C therapy substitutes.

vs. Amwell/SilverCloud

Spring Health; Lyra Health; Modern Health; Meru Health; Uwill (higher-ed niche); Woebot. Direct B2B clinical-platform substitutes.

Adjacent Disruptors

Hyperscalers: Amazon Clinic, CVS/Aetna virtual care. AI-native: Wysa, Woebot AI, ChatGPT-powered companions. In-person consolidators: LifeStance, Talkiatry — hybrid may eat pure-digital.

Pricing Benchmark

BetterHelp $60–$90/wk · Talkspace $70–$99/wk · Cerebral $100+/mo · SilverCloud (via payer) ~$25–$50 PMPM · Spring Health employer similar PMPM range.

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Benefits & *Limitations*

Clinical Non-Inferiority

2024 meta-analysis of 35 RCTs (n=4,827): non-inferior to in-person for depression and anxiety.

Expanded Access

HRSA: telehealth users 2.6x more likely to receive MH services; 71% user acceptability.

Severity Ceiling

Inadequate evidence for acute psychosis, severe mania, active suicidality, severe SUD.

Engagement & Safety Gaps

40-60% attrition in self-guided iCBT; crisis management harder; privacy risk (see FTC case).

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Key *Findings*

Clinically Valid

Digital mental health is clinically non-inferior for mild-to-moderate conditions, but not a full substitute for severe cases (e.g., psychosis, active suicidality).



Scale vs. Credibility

Teladoc leads in scale but faces commoditization, therapist variability, and trust issues (FTC case).
Amwell leads in clinical evidence but is constrained by slower enterprise growth.



Reimbursement decides

Reimbursement — not technology — is the single largest variable determining which companies survive through 2027. Stepped-care and measurement-based care become the standard, and that favors Amwell's structural design over Teladoc's.

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Strategic Recommendations for Teladoc

Pivot to Insurance

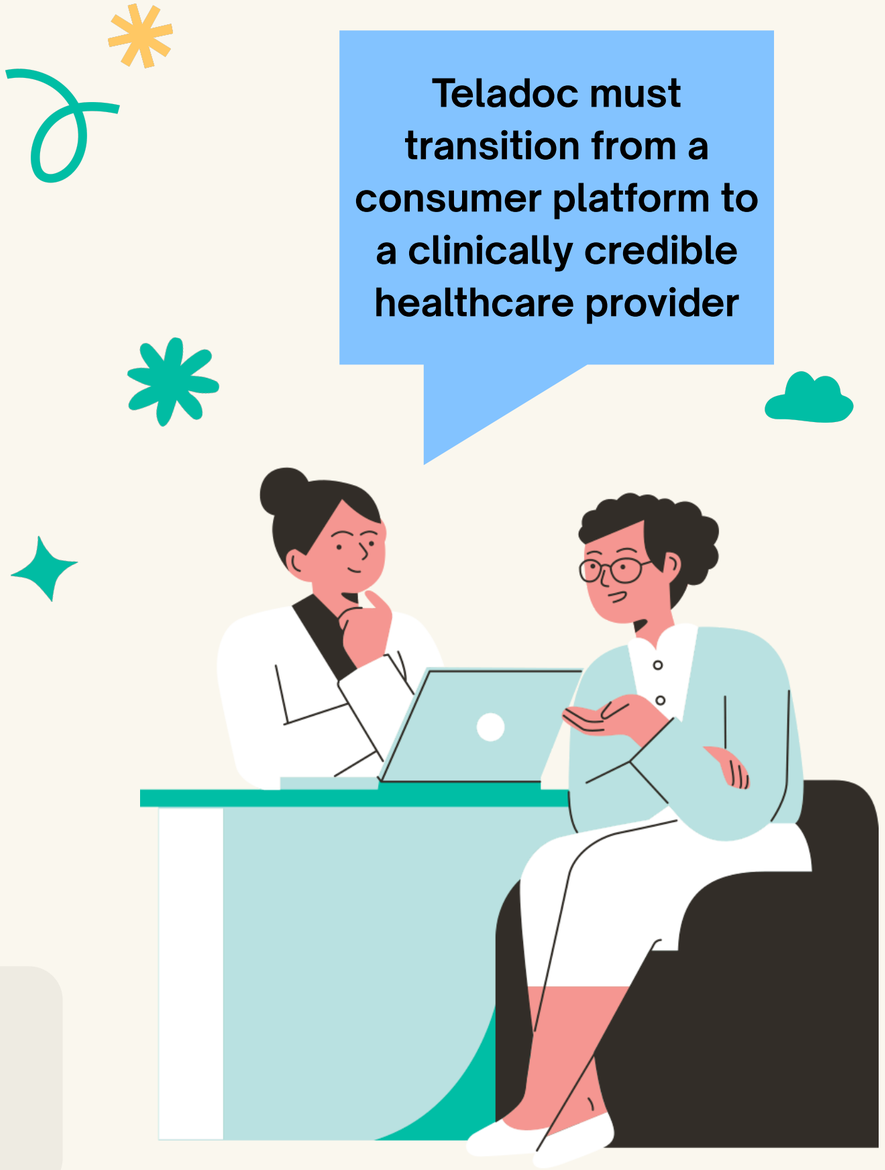
Shift from cash-pay competition to an insurance-driven model to improve pricing power and retention.

Build Clinical Credibility

Standardize PHQ-9/GAD-7 tracking and publish outcomes to rebuild trust and support reimbursement.

Acquire Digital CBT Capability

Add structured async CBT to close the clinical gap with evidence-based competitors like SilverCloud.



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Strategic Recommendations for Amwell

Productize Clinical Evidence

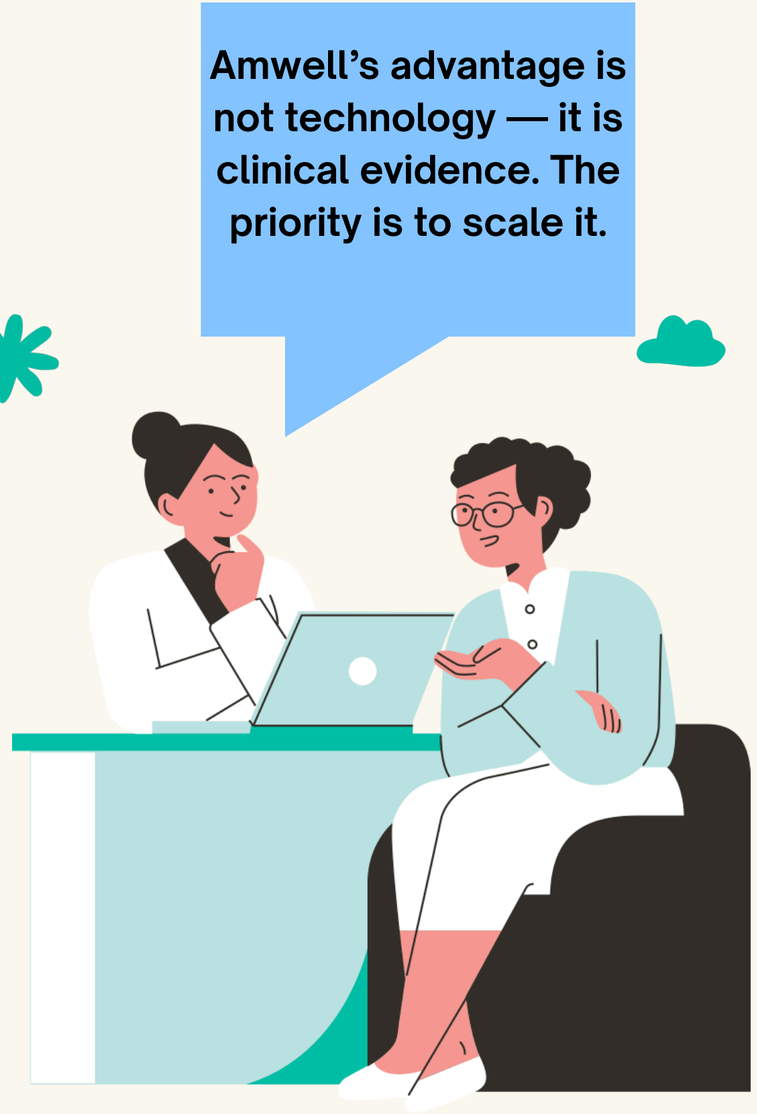
Turn SilverCloud outcomes (PHQ-9 / GAD-7) into a standardized industry benchmark to strengthen payer and employer adoption.

Expand Employer & Payer Channels

Scale distribution in the U.S. employer and insurance markets to accelerate growth beyond current enterprise constraints.

Diversify Government Exposure

Reduce reliance on DHA by expanding into Medicaid, VA, and other public-sector contracts.



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Investor Perspective

Short Term — Teladoc (TDOC)

- More near-term catalysts from insurance pivot, UpLift scaling, and partnerships.
- Higher momentum in the next 6–12 months.

Long Term — Amwell (AMWL)

- Structural advantage driven by clinical evidence and reimbursement alignment.
- Higher margin potential over time.

Key Insight

- Reimbursement — not technology — will determine long-term winners.



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Strategic Playbook

For Teladoc — Fix the Model

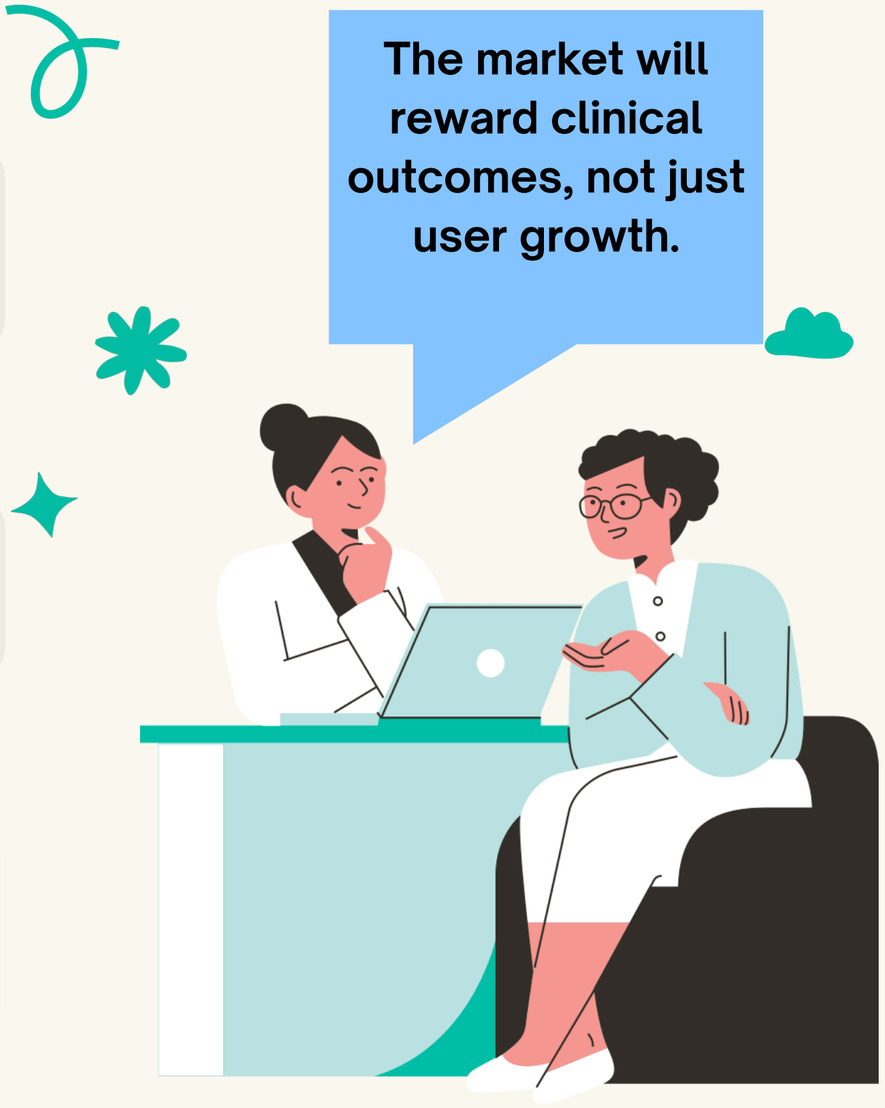
- Pivot to insurance
- Rebuild clinical credibility
- Acquire digital CBT capability

For Amwell — Scale the Advantage

- Productize clinical evidence
- Expand employer & payer channels
- Diversify government exposure

For Investors — Position Accordingly

- Short term: TDOC (momentum)
- Long term: AMWL (structural advantage)
- Stronger conviction: AMWL



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Thank You
Questions & Discussion



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Appendix

- TELADOC FY2023 → FY2024 → FY2025: Revenue $2.602B → $2.573B → $2.530B. BetterHelp segment $1.118B → $1.049B → $950M. Net loss $(220M) → $(1.007B w/ Livongo writedown) → $(200M). Cash $1.1B → $1.3B → $1.1B. Debt ~$1.5B convertible. Employees ~5,600 → 4,500 → 3,900.
- AMWELL FY2023 → FY2024 → FY2025: Revenue $259M → $315M → $245M. Net loss $(679M) → $(159M) → narrowing. Cash $260M → $245M → $219M. No long-term debt. Employees ~1,200 → 1,000 → 900.
- Sources: Teladoc 10-K and Amwell 10-K filings (FY2023, FY2024, FY2025) plus Q4 earnings releases.
- medRxiv (2024) 35-RCT meta-analysis n=4,827: SMD −0.03 depression, −0.06 anxiety telehealth vs in-person. Shaker et al (2023) JMIR Mental Health: telepsychiatry non-inferior. BMC Public Health (2024) 39 studies: 71% acceptability.
- VA Evidence Synthesis Program (2022): in-home VTC for PTSD clinically equivalent to clinic-based (low-strength evidence, 7 studies). Foundational to VA's tele-MH expansion.
- Richards & Richardson group (Trinity College Dublin): 30+ peer-reviewed RCTs across depression, anxiety, sleep, insomnia, chronic conditions. Foundation for NICE endorsement and Irish HSE deployment.
- Mind Share Partners (2025) Mental Health at Work Report. WHO (2025) World Mental Health Today. HRSA workforce projections. CB Insights Digital Health 50 (2025). Healthcare Dive, Fierce Healthcare ongoing coverage.
- Shared sensitive mental-health data — including users' answers to intake questions — with Facebook (Meta), Snapchat, Criteo, and Pinterest for advertising purposes. Violated explicit promises to users that data would be kept confidential.
- Case Number C-4796. Finalized July 2023. $7.8 million redress payment. 20-year data-sharing prohibition (runs through 2043). Affirmative-consent requirement. Biennial third-party privacy assessments.
- Structural constraint through 2043 limits BetterHelp's ability to use first-party data for AI personalization the way an unconstrained operator can. Compliance cost ~$1–3M/yr in audit overhead. Caps the AI moat Teladoc could build on its 2M+ user dataset.
- Endorsed by the UK National Institute for Health and Care Excellence as a recommended digital therapy for anxiety and depression. The only major digital MH product in this comparison with NICE endorsement.
- 30+ peer-reviewed randomized controlled trials across depression, anxiety, sleep, insomnia, and chronic conditions. Foundation for NICE endorsement. Core evidence for Amwell's clinical thesis.
- UK NHS Talking Therapies (IAPT) deployments across multiple regional trusts, integrated into first-line primary-care MH pathways. Irish HSE national digital CBT service: 55,000+ referrals processed. First national-scale EU deployment.
- 1M+ users globally. Approximately 80% show clinical improvement on PHQ-9 / GAD-7 after the program. 500+ health systems and organizations deployed worldwide; expanding U.S. employer/payer footprint 2024–2026.
- iCBT — internet-based Cognitive Behavioral Therapy. PHQ-9 — Patient Health Questionnaire-9 (depression screening 0–27). GAD-7 — Generalized Anxiety Disorder-7 (anxiety screening 0–21). Stepped care — matching treatment intensity to severity.
- What is BetterHelp's actual therapist retention rate (not publicly disclosed)? How much of Amwell's DHA revenue is at risk in the 2027 recompete? Does BetterHelp's insurance pivot cannibalize cash-pay or create net new patients?
- What does LLM-scale AI therapy do to unit economics by 2028? If Medicare tele-MH flexibilities lapse Dec 31, 2026, what is the per-company revenue impact in 2027? Can Converge attract third-party clinical apps at meaningful pace?
- Talkspace — smaller scale ($186M FY24). Spring Health, Lyra — private, limited disclosure. Cerebral — DOJ regulatory cloud. LifeStance — hybrid confounds digital framing. Headspace Health — primarily wellness. Woebot — not yet at comparable commercial scale.
- Measurement-based care — using validated outcome scales every session. Therapeutic alliance — clinical relationship quality, strongest predictor of outcomes. Async care — non-real-time messaging or program-based interaction.
- EAP — Employee Assistance Program. PMPM — Per Member Per Month (population billing). CAA 2023 — Consolidated Appropriations Act extending Medicare tele-MH. CPT codes 90791/90832/90834/90837 — psychotherapy billing.
- DHA — Defense Health Agency (Military Health System). MHPSA — Mental Health Professional Shortage Area. Ryan Haight Act — federal law on online prescribing of controlled substances. NICE — UK National Institute for Health and Care Excellence.